A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 July 2026

Dr May Wong

Dear Dr Wong,

Re: Mr Matthew Metcalf
DOB: 24/06/1988 Phone: 0411 010 394
Address: 21 Botherambo Street, RICHMOND, VIC, 3121
Email: mattmetcalf@live.com.au

Thank you for seeing Matthew, who is relocating to Sydney. He has been under my care for a number of years with a severe disorder of gut-brain interaction (previously severe functional gastrointestinal disorder), characterised by chronic abdominal pain, diarrhoea and marked functional impairment.

Matthew's symptoms began following an infective illness acquired while travelling in the Middle East. Early in the course of his illness he experienced severe abdominal pain and diarrhoea requiring hospitalisation overseas. At his worst, his symptoms were profoundly disabling, resulting in prolonged inability to work and significant reliance on opioid analgesia, although he has now been off opioids for several years.

The most successful treatment has been tricyclic antidepressant therapy. Initially, he achieved a dramatic improvement with **nortriptyline 100 mg nightly**, which was genuinely life-changing, allowing him to return to full-time employment after a prolonged period of disability. His principal adverse effect was tachycardia, although nortriptyline was otherwise well tolerated.

In 2024, following a recurrence of symptoms during a period of significant psychosocial stress (including divorce and relocation), he transitioned to **amitriptyline 100 mg nightly** after being unable to tolerate dothiepin. During this period he also became reliant on relatively high doses of loperamide, at one stage taking up to 30 tablets during severe exacerbations, although he recognised this as excessive. He subsequently experienced gradual clinical improvement with resolution of pain and progressive reduction in loperamide requirements.

By April 2025, Matthew had successfully reduced amitriptyline to **30 mg nightly** because of sedation and cognitive slowing. He had also substantially reduced his loperamide without significant deterioration in bowel symptoms. We agreed to discontinue the tricyclic completely and observe his progress, recognising that his symptoms remain somewhat unpredictable and are influenced by periods of significant life stress. Should pharmacological therapy again become necessary, I would favour **nortriptyline** over amitriptyline because it was previously effective and considerably less sedating.

Matthew's psychosocial background is relevant. He describes a difficult childhood with maternal mental illness and substance dependence, and I suspect longstanding hypervigilance contributes to central symptom amplification. He is highly intelligent, professionally successful and tends to approach both work and illness with considerable intensity. Given these factors, I referred him for psychiatric assessment in 2024, particularly as he himself recognised the potential benefit of psychological support. He also has a past history of ADHD treated in childhood.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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More recently, because of recurrent abdominal pain and altered bowel habit, I repeated his endoscopic assessment in July 2024. Gastroscopy demonstrated severe erosive gastritis involving the fundus and gastric body, with an otherwise normal oesophagus and duodenum. Gastric, oesophageal and duodenal biopsies were obtained. Colonoscopy showed a normal terminal ileum and colon apart from mild left-sided diverticulosis and small internal haemorrhoids. Random colonic biopsies were taken and there was no endoscopic evidence of inflammatory bowel disease.

Overall, I believe Matthew has a severe disorder of gut-brain interaction with visceral hypersensitivity and a strong relationship between symptom severity and psychological stress. He has responded remarkably well to tricyclic therapy in the past, and management has focused on gradually minimising medication while maintaining function. He has developed excellent insight into the biopsychosocial nature of his illness and has become progressively less reliant on medication over time.

Please do not hesitate to contact me if I can provide any further information regarding his previous management.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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